Thyrocare

Tests you can trust

Name : Jayhind Yadav(28Y/M)

Date : 08 Feb 2025

Test Asked : Fk Health Management Test

Report Status: Complete Report

100% NABL ACCREDITATION

First National Diagnostic Chain to have **100% of its Labs with NABL Accreditation#**

98% Reports released within **06 Hours** of sample reaching the lab+

9 out of 10 Doctors Trust that Thyrocare reports are **Accurate & Reliable***

1200+ Tests & Profiles

Temperature-Controlled Sample Logistics

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Abnormal Values Re-Checked Twice

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Your reports are digitally verifiable

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QR code will remain active for 30 days from report release date

Accredited by

NABL From 2005

ISO 9001: 2015 - From 2015

CAP From 2007

PROCESSED AT :
Thyrocare
Gargi Aura,420C-2nd Floor
IDA Scheme 114 Part 1,Dewas
Naka,Indore,Madhya Pradesh

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Thyrocare Technologies Limited, D-37/3, TTC MIDC, Turbhe, Navi Mumbai - 400 703 9870666333 wellness@thyrocare.com

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NAME : JAYHIND YADAV(28Y/M)
REF. BY : SELF
TEST ASKED : FK HEALTH MANAGEMENT TEST

HOME COLLECTION :
LNCT BHOPAL INDORE CAMPUS RAU PITHAMPUR RD, RAU, INDORE, MADHYA PRADESH LNCT BHOPAL INDORE CAMPUS

Report Availability Summary

Note: Please refer to the table below for status of your tests.

13 Ready | **0** Ready with Cancellation | **1** Processing | **0** Cancelled in Lab

TEST DETAILS	REPORT STATUS
FK HEALTH MANAGEMENT TEST	Partially Ready
LIPID PROFILE	Ready
T3-T4-USTSH	Ready
FASTING BLOOD SUGAR(GLUCOSE)	Ready
KIDPRO	Ready
VITAMIN B-12	Ready
IRON	Ready
LIVER FUNCTION TESTS	Ready
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	Ready
Lipoprotein (a) [Lp(a)]	Ready
APOLIPROTEIN RATIO	Ready
25-OH VITAMIN D (TOTAL)	Ready
ERYTHROCYTE SEDIMENTATION RATE (ESR)	Ready
HBA PROFILE	Ready
HEMOGRAM - 6 PART (DIFF)	Processing

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Summary Report

Tests outside reference range

TEST NAME	OBSERVED VALUE	UNITS	Bio. Ref. Interval.
CARDIAC RISK MARKERS			
Lipoprotein (a) [Lp(a)]	121.88	mg/dL	< 30
LIPID			
HDL / LDL RATIO	0.27	Ratio	> 0.40
LDL / HDL RATIO	3.7	Ratio	1.5-3.5
LDL CHOLESTEROL - DIRECT	174	mg/dL	< 100
NON-HDL CHOLESTEROL	221	mg/dL	< 160
TC/ HDL CHOLESTEROL RATIO	5.7	Ratio	3 - 5
TOTAL CHOLESTEROL	268	mg/dL	< 200
TRIG / HDL RATIO	5.71	Ratio	< 3.12
TRIGLYCERIDES	269	mg/dL	< 150
VLDL CHOLESTEROL	53.7	mg/dL	5 - 40
LIVER			
BILIRUBIN (INDIRECT)	0.92	mg/dL	0-0.9
PROTEIN - TOTAL	8.6	gm/dL	5.7-8.2
SERUM GLOBULIN	3.8	gm/dL	2.5-3.4
OTHER COUNTS			
MEAN CORP.HEMO.CONC(MCHC)	30.4	g/dL	31.5-34.5
MEAN CORPUSCULAR HEMOGLOBIN(MCH)	23.4	pq	27.0-32.0
MEAN CORPUSCULAR VOLUME(MCV)	76.9	fL	83.0-101.0
RED CELL DISTRIBUTION WIDTH (RDW-CV)	16.2	%	11.6-14
TOTAL RBC	6.33	X 10^6/µL	4.5-5.5
RENAL			
URIC ACID	8.5	mg/dL	4.2 - 7.3
THYROID			
TSH - ULTRASENSITIVE	5.18	µIU/mL	0.55-4.78
VITAMIN			
25-OH VITAMIN D (TOTAL)	< 4.2	ng/mL	30-100

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

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TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c - (HPLC)			
	H.P.L.C	5.7	%

Bio. Ref. Interval. :

Bio. Ref. Interval.: As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal	Below 6.5% : Good Control
5.7% - 6.4% : Prediabetic	6.5% - 7% : Fair Control
>=6.5% : Diabetic	7.0% - 8% : Unsatisfactory Control
	>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	CALCULATED	117	mg/dL

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

Sample Collected on (SCT) :08 Feb 2025 08:58
Sample Received on (SRT) : 08 Feb 2025 13:08
Report Released on (RRT) : 08 Feb 2025 16:01
Sample Type : EDTA Whole Blood
Labcode : 0802041150/DI191
Barcode : CK344876

Dr Bhavya MD(Path)

Scan QR code to verify authenticity of reported results; active for 30 days from release time.

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TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	14.8	g/dL	13.0-17.0
Hematocrit (PCV)	CPH Detection	48.7	%	40.0-50.0
Total RBC	**HF & EI**	**6.33**	**X 10^6/µL**	**4.5-5.5**
Mean Corpuscular Volume (MCV)	**Calculated**	**76.9**	**fL**	**83.0-101.0**
Mean Corpuscular Hemoglobin (MCH)	**Calculated**	**23.4**	**pq**	**27.0-32.0**
Mean Corp.Hemo. Conc (MCHC)	**Calculated**	**30.4**	**g/dL**	**31.5-34.5**
Red Cell Distribution Width - SD (RDW-SD)	Calculated	40.9	fL	39-46
Red Cell Distribution Width (RDW - CV)	**Calculated**	**16.2**	**%**	**11.6-14**
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	4.83	X 10^3 / µL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	53.4	%	40-80
Lymphocytes Percentage	Flow Cytometry	38.3	%	20-40
Monocytes Percentage	Flow Cytometry	5.4	%	2-10
Eosinophils Percentage	Flow Cytometry	2.1	%	1-6
Basophils Percentage	Flow Cytometry	0.6	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.2	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.1	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	2.58	X 10^3 / µL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	1.85	X 10^3 / µL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.26	X 10^3 / µL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.03	X 10^3 / µL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.1	X 10^3 / µL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.01	X 10^3 / µL	0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10^3 / µL	0.0-0.5
PLATELET COUNT	HF & EI	150	X 10^3 / µL	150-410
Others				
ERYTHROCYTE SEDIMENTATION RATE (ESR)	MODIFIED WESTERGREN	12	mm / hr	0 - 15

Clinical history is asked for all the relevant abnormalities detected and in absence / failure of receiving of clinical history, results are rechecked twice and released. Advised clinical correlation.
Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)
(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height**)**

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TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	**C.L.I.A**	**< 4.2**	**ng/mL**

Bio. Ref. Interval. :-

DEFICIENCY : <20 ng/ml || INSUFFICIENCY : 20-<30 ng/ml
SUFFICIENCY : 30-100 ng/ml || TOXICITY : >100 ng/ml

Clinical Significance:

Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health. Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome. Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml.

Kit Validation Reference: Holick MF. Vitamin D Deficiency. N Engl J Med. 2007;357:266–81.

Please correlate with clinical conditions.
Method:- Fully Automated Chemi Luminescent Immuno Assay

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Sample Type : SERUM
Labcode : 0802075597/DI191
Barcode : CM710098

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TEST NAME	TECHNOLOGY	VALUE	UNITS
APOLIPOPROTEIN - A1 (APO-A1)	IMMUNOTURBIDIMETRY	131	mg/dL

Bio. Ref. Interval. :
Male : 86 - 152
Female : 94 - 162
Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY – BECKMAN COULTER

APOLIPOPROTEIN - B (APO-B)	IMMUNOTURBIDIMETRY	142	mg/dL

Bio. Ref. Interval. :
Male : 56 - 145
Female : 53 - 138
Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY – BECKMAN COULTER

APO B / APO A1 RATIO (APO B/A1)	CALCULATED	1.1	Ratio

Bio. Ref. Interval. :

Male : 0.40 - 1.26
Female : 0.38 - 1.14

Clinical Significance :

- Apolipoprotein B is a more potent and independent predictor of Coronary artery disease (CAD) than LDL Cholesterol.
- Apolipoprotein A1 is one of the apoproteins of HDL and is inversely related to risk of CAD.
- The Apolipoprotein studies help in monitoring risk of restenosis in patients with myocardial infarction, Coronary bypass surgery etc.
- An increased ratio of Apo B to A1 beyond the defined normal range is indicative of CAD risk.
- All results have to be interpreted in Conjunction with clinical history and other findings.

Method : Derived from serum Apo A1 and Apo B values

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	IMMUNOTURBIDIMETRY	2.55	mg/L

Bio. Ref. Interval. :-

< 1.00 - Low Risk
1.00 - 3.00 - Average Risk
>3.00 - 10.00 - High Risk
> 10.00 - Possibly due to Non-Cardiac Inflammation

Disclaimer: Persistent unexplained elevation of HSCRP >10 should be evaluated for non-cardiovascular etiologies such as infection , active arthritis or concurrent illness.

Clinical significance:
High sensitivity C- reactive Protein (HSCRP) can be used as an independent risk marker for the identification of Individuals at risk for future cardiovascular Disease. A coronary artery disease risk assessment should be based on the average of two hs-CRP tests, ideally taken two weeks apart.

Kit Validation Reference:
1.Clinical management of laboratory date in medical practice 2003-3004, 207(2003).
2.Tietz : Textbook of Clinical Chemistry and Molecular diagnostics :Second edition :Chapter 47:Page no.1507- 1508.

Please correlate with clinical conditions.
Method:- FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER

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TEST NAME	TECHNOLOGY	VALUE	UNITS
VITAMIN B-12	C.L.I.A	239	pg/mL

Bio. Ref. Interval. :-

Normal : 211 - 911 pg/ml

Clinical significance :
Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):5.0%, Inter assay (%CV):9.2 %;Sensitivity:45 pg/ml

Kit Validation reference:
Chen IW, Sperling MI, Heminger LA. Vitamin B12. In: Pesce AJ, Kaplan LA, eds. Methods in Clinical Chemistry. St. Louis: CV Mosby; 1987:569–73.

Please correlate with clinical conditions.
Method:- COMPETITIVE CHEMI LUMINESCENT IMMUNO ASSAY

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TEST NAME	TECHNOLOGY	VALUE	UNITS
Lipoprotein (a) [Lp(a)]	**IMMUNOTURBIDIMETRY**	**121.88**	**mg/dL**

Bio. Ref. Interval. :-

Adults : < 30.0 mg/dl

Clinical Significance:
Determination of LPA may be useful to guide management of individuals with a family history of CHD or with existing disease. The levels of LPA in the blood depends on genetic factors; The range of variation in a population is relatively large and hence for diagnostic purpose, results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

Specifications:
Precision %CV :- Intra assay %CV- 4.55% , Inter assay %CV-0.86 %

Kit Validation Reference:
Tietz NW,Clinical Guide to Laboratory Tests Philadelphia WB. Saunders 1995 : 442-444

Please correlate with clinical conditions.
Method:- LATEX ENHANCED IMMUNOTURBIDIMETRY

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL CHOLESTEROL	**PHOTOMETRY**	**268**	**mg/dL**	**< 200**
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	47	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**174**	**mg/dL**	**< 100**
TRIGLYCERIDES	**PHOTOMETRY**	**269**	**mg/dL**	**< 150**
TC/ HDL CHOLESTEROL RATIO	**CALCULATED**	**5.7**	**Ratio**	**3 - 5**
TRIG / HDL RATIO	**CALCULATED**	**5.71**	**Ratio**	**< 3.12**
LDL / HDL RATIO	**CALCULATED**	**3.7**	**Ratio**	**1.5-3.5**
HDL / LDL RATIO	**CALCULATED**	**0.27**	**Ratio**	**> 0.40**
NON-HDL CHOLESTEROL	**CALCULATED**	**221**	**mg/dL**	**< 160**
VLDL CHOLESTEROL	**CALCULATED**	**53.7**	**mg/dL**	**5 - 40**

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
ALKALINE PHOSPHATASE	PHOTOMETRY	101.5	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	1.07	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.15	mg/dL	< 0.3
BILIRUBIN (INDIRECT)	**CALCULATED**	**0.92**	**mg/dL**	**0-0.9**
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	37.3	U/L	< 55
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	29.4	U/L	< 35
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	16.8	U/L	< 45
SGOT / SGPT RATIO	CALCULATED	1.75	Ratio	< 2
PROTEIN - TOTAL	**PHOTOMETRY**	**8.6**	**gm/dL**	**5.7-8.2**
ALBUMIN - SERUM	PHOTOMETRY	4.8	gm/dL	3.2-4.8
SERUM GLOBULIN	**CALCULATED**	**3.8**	**gm/dL**	**2.5-3.4**
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.26	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Vanadate Oxidation
BILD - Vanadate Oxidation
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

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TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON	PHOTOMETRY	79	µg/dL

Bio. Ref. Interval. :
Male : 65 - 175
Female : 50 - 170
Method : Ferrozine method without deproteinization

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	8	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	0.75	mg/dL	0.72-1.18
BUN / SR.CREATININE RATIO	CALCULATED	10.67	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	17.12	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	22.83	Ratio	< 52
CALCIUM	PHOTOMETRY	10.01	mg/dL	8.8-10.6
URIC ACID	**PHOTOMETRY**	**8.5**	**mg/dL**	**4.2 - 7.3**

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method

Sample Collected on (SCT) : 08 Feb 2025 08:58
Sample Received on (SRT) : 08 Feb 2025 13:08
Report Released on (RRT) : 08 Feb 2025 16:22
Sample Type : SERUM
Labcode : 0802075597/DI191
Barcode : CM710098

Dr Bhavya MD(Path)

NAME : JAYHIND YADAV(28Y/M)
REF. BY : SELF
TEST ASKED : FK HEALTH MANAGEMENT TEST

HOME COLLECTION :
LNCT BHOPAL INDORE CAMPUS RAU PITHAMPUR RD, RAU, INDORE, MADHYA PRADESH LNCT BHOPAL INDORE CAMPUS

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	C.L.I.A	121	ng/dL	60-200
TOTAL THYROXINE (T4)	C.L.I.A	8.1	µg/dL	4.5-12
TSH - ULTRASENSITIVE	**C.L.I.A**	**5.18**	**µIU/mL**	**0.55-4.78**

Comments : ***

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3 - Competitive Chemi Luminescent Immuno Assay
T4 - Competitive Chemi Luminescent Immuno Assay
USTSH - Third Generation Ultrasensitive Chemi Luminescent Immuno Assay

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

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TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	125	mL/min/1.73 m2

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.
Method:- CKD-EPI Creatinine Equation

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NAME : JAYHIND YADAV(28Y/M)
REF. BY : SELF
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TEST NAME	TECHNOLOGY	VALUE	UNITS
FASTING BLOOD SUGAR(GLUCOSE)	PHOTOMETRY	93.3	mg/dL

Bio. Ref. Interval. :-

As per ADA Guideline: Fasting Plasma Glucose (FPG)	
Normal	70 to 100 mg/dl
Prediabetes	100 mg/dl to 125 mg/dl
Diabetes	126 mg/dl or higher

Note :
The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

Please correlate with clinical conditions.

Method:- GOD-PAP METHOD

~~ End of report ~~

Sample Collected on (SCT) : 08 Feb 2025 08:58
Sample Received on (SRT) : 08 Feb 2025 13:08
Report Released on (RRT) : 08 Feb 2025 13:43
Sample Type : FLUORIDE PLASMA
Labcode : 0802075567/DI191
Barcode : CO404991

Dr Bhavya MD(Path)

Scan QR code to verify authenticity of reported results; active for 30 days from release time.

CONDITIONS OF REPORTING

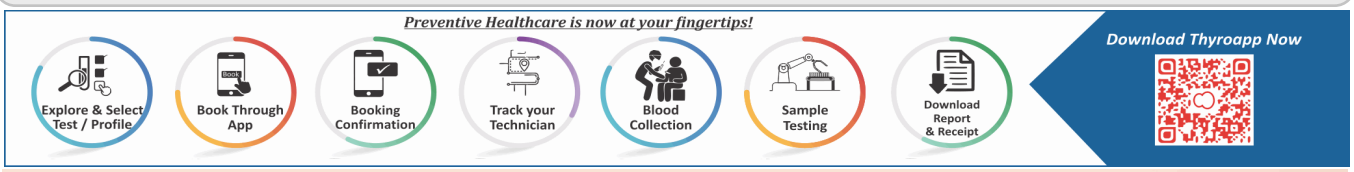
- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Neither Thyrocare, nor its employees/representatives assume: (a) any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report, (b) any claims of any nature whatsoever arising from or relating to the performance of the requested tests as well as any claim for indirect, incidental or consequential damages. The total liability, in any case, of Thyrocare shall not exceed the total amount of invoice for the services provided and paid for.
- Thyrocare Discovery video link :- https://youtu.be/nbdYeRgYyQc

EXPLANATIONS

- Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range** - Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- Values out of reference range requires reconfirmation before starting any medical treatment.
- Retesting is needed if you suspect any quality shortcomings.
- Testing or retesting should be done in accredited laboratories.
- For suggestions, complaints, clinical support or feedback, write to us at **customersupport@thyrocare.com** or call us on **022-3090 0000**



*+T&C Apply, #As on 5th December 2024, *As per a survey on doctors' perception of laboratory diagnostics (IJARIIT,2023)*